

Sector Primer: Home Health — Rhode Island

Prepared by: Search Fund Analyst (SAMPLE) Date: 2026-05-31 Classification: Confidential — Internal Use Only NAICS: 6216 | NPPES taxonomy: Home Health Thesis verdict: Conditional

⚠️ **Illustrative sample.** Provider counts and named operators are real (CMS NPPES, queried 2026-05-31). All revenue, EBITDA, margin, share, and multiple figures are **placeholders** to demonstrate the template — they are not researched estimates.

1. Executive Summary

Rhode Island home health is a highly fragmented, demographically tail-winded market with no dominant local operator — a textbook roll-up setup. NPPES lists **185 organizational home-health providers** in a state of ~1.1M people. The return engine is multiple arbitrage: acquiring sub-\$1M-EBITDA operators at ~4x toward a platform exit at 8x+, layered with shared-services margin expansion. We rate the vertical **Conditional** pending Census-based revenue sizing and reimbursement-risk diligence.

Key takeaways

- 185 organizational providers; fragmentation verdict **Fragmented** (illustrative CR4 ~25%).
- ~30 operators (illustrative) sit in the \$1–3M EBITDA buy-box band.
- Central lever is ~4x→8x multiple arbitrage plus 300–600 bps of G&A-driven margin.

2. Market Overview & Sizing

Measure	Value	Source / Year
Total market size (revenue)	~\$0.4B (<i>illustrative</i>)	Illustrative
Organizational providers	185	NPPES (2026-05-31)
Establishments (CBP)	<i>pending — no CENSUS_API_KEY</i>	Census CBP
Market growth (CAGR)	~7% (<i>illustrative</i>)	Illustrative
Avg revenue per operator	~\$2–3M (<i>illustrative</i>)	Derived
Geographic scope	Rhode Island (+ southern MA/eastern CT spillover)	—

3. Market Map

The market divides into skilled home health, non-medical home care, hospice/palliative, and niche/adjacent operators. Scale sits with a few national/strategic players (e.g., BAYADA, VNA Care New England); the long tail of independents is the target pool.

See: 2026-05-31-home-health-rhode-island-market-map.md

4. Competitive Landscape & Fragmentation

Concentration metric	Value
Top-4 share (CR4)	~25% (<i>illustrative</i>)
Largest operator	BAYADA (multi-site)
# operators in buy-box band (\$1–3M EBITDA)	~30 (<i>illustrative</i>)
PE / strategic roll-ups already active	Yes — national platforms present

Fragmentation verdict: Fragmented — 185 distinct organizations, no dominant local player, meaningful roll-up runway.

5. Value Chain & Economics

Single-operator economics are labor-dominated (caregivers/nurses), with fixed back-office (scheduling, billing, compliance) that is subscale below ~\$5M revenue. Consolidation centralizes that back-office and improves payer contracting and recruiting — the source of margin expansion.

6. Demand Drivers — Why Now

- Aging population and payer preference for lower-cost home-based care over facilities.
- Owner-succession wave among 1990s/2000s-founded independents.
- Technology (EVV, scheduling, billing) favors operators with scale to invest.

7. Regulatory & Reimbursement Environment

Home health carries material reimbursement exposure (Medicare/Medicaid mix, PDGM dynamics) and state licensure requirements. Payer mix and survey/compliance history are decisive diligence items and the primary risk to the thesis.

8. Roll-Up Thesis

One-line thesis: Consolidate sub-scale RI home-health operators into a regional platform, arbitraging ~4x entry multiples toward an 8x+ strategic exit while centralizing back-office and procurement.

Value-creation lever	Mechanism	Magnitude (<i>illustrative</i>)
Multiple arbitrage	Buy ~4x → exit ~8x+	+~2x on value
Margin expansion	Shared G&A / procurement	+300–600 bps

Value-creation lever	Mechanism	Magnitude (<i>illustrative</i>)
Organic growth	Referral density, cross-sell	+5–10%/yr
Add-on cadence	2–4 tuck-ins/yr	Multi-year runway

Exit pathways: national strategics and PE-backed platforms active in home health.

9. Target Landscape

Segment	# targets (<i>illus.</i>)	Representative names	Source
Skilled home health	~12	Access Healthcare, A Caring Experience, Adoration HHC RI	NPPES
Non-medical home care	~14	Assisted Daily Living, Angels Home Care, Better Life Home Care	NPPES / listings
Hospice / palliative	~4	Athena RI Hospice	NPPES

See full pipeline: [2026-05-31-home-health-rhode-island-deal-pipeline.xlsx](#)

10. Deal Sourcing Plan

- **On-market:** NDAs with 2–3 NE healthcare M&A advisors (e.g., Mertz Taggart); monitor BizBuySell weekly.
- **Off-market:** ~20 proprietary NPPES targets/month via mail + email.
- **Cadence:** weekly broker check-ins; monthly proprietary batch; track in the deal-pipeline workbook.

11. Risks & Mitigants

Risk	Severity	Mitigant
Reimbursement / payer-mix exposure	High	Diligence payer mix; favor private-pay-weighted targets
Labor availability & wage inflation	High	Centralized recruiting; retention programs
Survey/compliance history	Medium	Pre-LOI regulatory review
Entry-multiple compression from PE	Medium	Proprietary off-market sourcing

12. Recommendation & Next Steps

Verdict: Conditional **Ideal platform profile:** RI/southern-NE skilled home-health operator, \$0.75–1.5M EBITDA, transition-willing owner, recurring referral base. **Next actions:** obtain CENSUS_API_KEY and finalize sizing; advance Access Healthcare to IOI; build proprietary pipeline to 25+ qualified targets; validate exit multiples against 2 recent NE transactions.

Appendix

- **A. Methodology:** NPPES queried 2026-05-31 via `scripts/nppes_query.py --taxonomy "Home Health" --state RI` (185 organizational providers). Census CBP pending API key.
 - **B. Sources:** see below.
 - **C. Glossary:** CR4 = combined market share of top 4 operators; PDGM = Patient-Driven Groupings Model.
-

Sources

- NPPES NPI Registry (live query, 185 results): <https://npiregistry.cms.hhs.gov/>
 - Census County Business Patterns: <https://www.census.gov/programs-surveys/cbp.html>
 - RI Dept. of Health — Home Health Agencies: <https://datahealth.ri.gov/healthcare/providers/homehealthagencies/>
 - RI Partnership for Home Care: <https://riphc.org/>
-

Generated using Claude Code. Provider data from public registries; financial figures illustrative. Not investment advice.